AIIMS Mangalagiri Medical Reports

Plain-language explanation of the uploaded clinic notes and laboratory results | Report dates visible: 5–13 October 2026

Important: This document explains what can be read from the photographs. Some handwritten drug names, doses and instructions are unclear. It is not an official AIIMS report, diagnosis, or replacement for the treating gynaecologist/pharmacist confirming the original prescription.

1. Overall summary

The main confirmed laboratory finding is **anaemia**: haemoglobin is 9.7 g/dL, below the laboratory range of 12–15 g/dL. The red blood cells are smaller than usual (MCV 74.5 fL) and contain less haemoglobin per cell (MCH 22 pg). This pattern often occurs with iron deficiency, and the reported heavy menstrual bleeding is a plausible source of iron loss. Iron deficiency has not been proven by the CBC alone; ferritin/iron studies and clinical review can confirm it.

The reported symptoms—tiredness, weakness and dizziness—can occur with anaemia. Platelet count and total white blood cell count are within the laboratory ranges. TSH is 1.50 mIU/L, within the adult reference interval printed on the report.

Priority: heavy bleeding plus dizziness/weakness needs timely clinical assessment. If bleeding is heavy now, or there is fainting, chest pain, breathlessness at rest, a racing heartbeat with marked weakness, or difficulty standing, seek emergency care.

2. Laboratory results explained

Test	Result	Meaning
Haemoglobin (Hb)	**9.7 g/dL** (low)	Confirms anaemia. Can contribute to fatigue, weakness, dizziness, shortness of breath or palpitations.
RBC count	4.40 million/µL	Within the printed range; does not rule out iron deficiency.
Haematocrit (HCT)	32.8% (slightly low)	Proportion of blood occupied by red cells; consistent with anaemia.
MCV	**74.5 fL (low)**	Red cells are smaller than usual (microcytosis); iron deficiency is a common cause.
MCH	**22.0 pg (low)**	Less haemoglobin per red cell; often seen with iron deficiency.
MCHC	29.6 g/dL (low)	Red cells have a lower haemoglobin concentration than the reference range.
RDW-CV	**16.2% (high)**	Red-cell sizes vary more than usual; can support iron deficiency but is not diagnostic alone.
TLC / white cells	7.60 × 10^3/µL	Within the printed range of 4–10.
Platelets	361 × 10^3/µL	Within the printed range of 150–450.
TSH	1.50 mIU/L	Within the report's adult reference range; this test alone does not suggest an obvious thyroid abnormality.

Other differential white-cell counts shown on the CBC are broadly within their printed ranges. Basophils/immature granulocytes are at or near the upper limit in some entries; these small findings are not the main issue compared with the anaemia and bleeding history.

3. What the doctor appears to have documented

The clinic cards identify the visit as an Obstetrics & Gynaecology appointment. The handwritten notes appear to record a history of menstrual cycles/bleeding and an examination. The images are not clear enough to transcribe every abbreviation with confidence, so the following is a cautious reading, not a verbatim medical record:

• The history seems to mention menstrual-cycle details, including duration/flow, and a statement resembling “no other complaints” in one section. This should be confirmed with the treating clinician because the handwriting and abbreviations are ambiguous.

• The later note appears to mention “AUB evaluation.” AUB commonly means **abnormal uterine bleeding**—bleeding that is unusually heavy, prolonged, irregular, or occurs outside the expected pattern. It is a description of the symptom, not by itself the final cause.

• The note appears to record that she is around 47 years old. At this age, persistent or newly worsening heavy bleeding warrants assessment for causes such as fibroids, polyps, adenomyosis, hormonal changes around perimenopause, and—when appropriate—problems of the uterine lining.

• The blood pressure, height and weight fields on the printed card appear blank. I cannot reliably identify a measured blood pressure from these photographs. No high or low blood-pressure diagnosis should be inferred from this paperwork.

4. Medicines: what can and cannot be read

Do not use this table as a dosing instruction. Handwritten medicine names and dosing marks are partly illegible. Confirm each item with the original prescription, AIIMS pharmacy, or treating doctor before taking it. Do not add, stop, or alter medicines based on this interpretation.

Possible item in note	What it may be for	Confidence / safety note
Iron/folic-acid preparation (handwriting appears to include “IFA” and a strength, but not fully readable)	Iron replaces iron lost through menstrual bleeding; folic acid supports red-cell production when needed.	Likely purpose only. Exact brand, elemental iron dose, frequency and duration must be confirmed. Since she is already taking prescribed iron, follow that verified prescription.
Vitamin C (possible “Vit C” entry)	May be prescribed alongside iron; vitamin C can improve absorption of non-haem iron in some settings.	The exact drug and dose are not sufficiently clear to confirm.
Calcium + vitamin D (appears in later note)	Supports calcium/vitamin D intake or bone health when indicated; it does not treat menstrual blood loss or anaemia directly.	The strength and directions are unclear. Calcium can interfere with iron absorption if taken together; ask the prescriber/pharmacist how to space them.
Doxycycline 100 mg (appears written)	An antibiotic used for selected bacterial infections; it is not a general treatment for anaemia or heavy periods.	The indication, duration and exact directions cannot be verified from the image. Use only if the treating clinician confirms it was prescribed for her and explains why.
Metrogyl / metronidazole 400 mg (appears written)	An antimicrobial used for certain bacterial/parasite infections; not an anaemia treatment.	Exact schedule and indication are uncertain. Confirm with the doctor/pharmacist. Avoid alcohol during treatment and for the period specified by the pharmacist after the last dose.
Pantoprazole 40 mg (name appears similar to “Pantop 40”)	Reduces stomach acid and may be used for acid symptoms or stomach protection in selected situations.	Exact directions are unclear. It does not stop uterine bleeding or correct iron deficiency itself.
A tablet that may read “Trapic/Tranexamic acid 500 mg” (uncertain)	If this is tranexamic acid, it is used in some patients to reduce heavy menstrual bleeding by helping clots remain stable.	**Do not assume this identification or take it from this reading.** Confirm the drug and directions. Tranexamic acid is not suitable for everyone, especially some people with a history of blood clots or particular risk factors.

The first prescription page also appears to list additional blood/urine tests or investigations, but not all names are legible. A later note seems to mention blood group, HIV, HBsAg, HCV and urine testing, as well as an X-ray/mammogram and review of reports. These are tentative readings: ask the clinic which tests are actually ordered and why. A mammogram is a breast-imaging test and is separate from investigating heavy menstrual bleeding.

5. Blood pressure and menstrual bleeding: understanding “low, normal, high, short and long”

Blood pressure: I cannot see a reliable blood-pressure measurement on the uploaded clinic card, so no one can conclude from these images that her BP was high or low.

Category for most adults	Approximate reading	Meaning
Low blood pressure	Often below 90/60 mmHg	May be normal for some people, but dizziness, fainting, confusion or weakness with a low reading needs clinical assessment—especially during bleeding.
Usual/normal	Below 120/80 mmHg is generally considered normal	A single reading does not rule out anaemia or blood loss.
Elevated / high	Repeated readings at or above 130/80 may be considered high under some guidelines; 140/90 or above is high in many clinical settings	Needs repeat measurement and clinician interpretation. One reading alone is not enough to diagnose chronic hypertension.

Menstrual bleeding: “Short” or “long” refers to the number of days; “light” or “heavy” refers to the amount of blood. Heavy bleeding can mean needing to change a pad/tampon every 1–2 hours, bleeding longer than 7 days, passing large clots, bleeding through clothes/bedding, or bleeding that disrupts daily activities. The history of heavy bleeding/large clots plus Hb 9.7 makes this important to discuss with the gynaecologist.

If she is soaking a pad each hour for two or more hours, especially with dizziness, faintness, chest pain or breathlessness, seek emergency care. Do not rely on a home BP reading to decide whether severe bleeding is safe.

6. Recommended next steps

Today / within 24 hours: Contact the gynaecology clinic if dizziness or weakness continues, bleeding is active, or the bleeding is recurring. If severe symptoms or heavy ongoing bleeding occur, go to emergency care.

Ask the clinician about:

1. Repeat CBC and serum ferritin (iron stores); additional iron studies may be appropriate. The clinician should decide timing based on symptoms and treatment.

2. Assessment of menstrual blood loss and the cause. Depending on history/examination, this may include pelvic examination, pelvic ultrasound, pregnancy testing if biologically possible, and further evaluation of the uterine lining because of her age and bleeding pattern.

3. A clear plan to control bleeding and a clear iron-treatment plan. Ask when haemoglobin should be rechecked and what to do if it does not rise.

4. Review of other possible causes if iron deficiency is not confirmed or response is poor, including gastrointestinal blood loss, nutritional deficiency, or inherited red-cell conditions.

5. Bring all medicine strips, the original prescription, and all reports. Ask the pharmacist to write the medicine name, dose, timing, duration and whether it should be taken with food.

7. How to take prescribed iron safely (general guidance)

• Take only the dose prescribed by her clinician. Iron commonly causes constipation, nausea, abdominal discomfort and dark stools. Keep iron away from children because overdose can be dangerous.

• Tea, coffee, calcium supplements and some antacids can reduce iron absorption. Ask the pharmacist how to space iron from calcium and other medicines; do not change the prescription yourself.

• Iron treatment generally takes weeks to improve haemoglobin and longer to replenish iron stores. Ongoing heavy bleeding can outpace replacement, so treating the bleeding matters too.

• If tablets are not tolerated, are not working, or anaemia is more significant than expected, the doctor may consider another formulation or intravenous iron after assessment.

8. Follow-up checklist

Item to confirm	Status / notes
Is heavy bleeding happening now? How many pads per hour?	_____
Any fainting, chest pain, breathlessness at rest or racing heartbeat?	_____
Exact iron product, elemental iron dose, schedule and start date	_____
Ferritin / iron studies ordered or completed?	_____
Gynaecology plan for cause and control of bleeding	_____
Date for repeat CBC and clinician review	_____
Exact medicine names/doses confirmed by AIIMS pharmacist	_____

9. Bottom line

The CBC confirms anaemia (Hb 9.7 g/dL) with small, relatively haemoglobin-poor red cells; iron deficiency from heavy menstrual bleeding is a leading possibility, but should be confirmed. The platelet and white-cell counts are within their printed ranges, and TSH is within the adult reference interval. The most important actions are to assess the severity of current bleeding, identify and treat its cause, verify the exact prescription, and monitor the response to iron.

Emergency reminder: heavy bleeding with fainting, chest pain, breathlessness at rest, severe weakness, or soaking a pad each hour for two or more hours warrants urgent emergency assessment.

Sources and limitations: This explanation is based on the uploaded photographs, including AIIMS Mangalagiri CBC and TSH reports dated 5 October 2026 and handwritten clinic notes dated around 6–13 October 2026. Medication names and directions are partly illegible and have been labelled as uncertain. This document does not establish a diagnosis or replace the treating clinician.